

Major objectives

- To confirm the clinical diagnosis of **pemphigus**.
- To identify risk factors, severity factors, and potential comorbidities through detailed history and initial clinical evaluation.
- To determine the type of initial involvement (skin, mucosa) and assess its extent.
- To evaluate prognosis based on patient age and overall general health.
- To measure the extent and distribution of lesions using validated scoring systems: **Autoimmune Bullous Skin Intensity and Severity Score (ABSIS)** or **Pemphigus Disease Area Index (PDAI)**.
- To assess the impact on quality of life using dermatology-specific tools such as **DLQI (Dermatology Life Quality Index)** and autoimmune blistering disease-specific instruments (**ABQOL, TABQOL**).
- To initiate prompt treatment aimed at achieving disease control and complete remission as early as possible.

Professions involved

The management of pemphigus should be led by an experienced **dermatologist**, typically based in a tertiary referral center, specialized clinic, or within a recognized expert network.

Supportive roles may be provided by the following healthcare professionals:

- Consultant dermatologist in general practice
- The patient's general practitioner
- Specialists relevant to the patient's condition and comorbidities, including:
- Internists
- Cardiologists

- Stomatologists
- Ophthalmologists
- Otorhinolaryngologists
- Gastroenterologists
- Gynaecologists
- Urologists
- Proctologists
- Rheumatologists
- Oncologists
- Psychologists
- Health nurses, particularly for home care in elderly or disabled patients with residual mucosal or skin lesions post-hospitalization
- Dietician and physiotherapist
- Nurse specialist/practitioner

It is recommended that:

- A dermatologist with expertise in pemphigus oversees the initial treatment plan.
- Other healthcare professionals are integrated into the care team based on disease involvement, severity, comorbidities, and complications.

Clinical examination

Medical history

It is recommended to document the following:

- Time of first onset of symptoms.
- Functional symptoms: pain, pruritus, severity of dysphagia, ocular and ENT symptoms, dysuria, anogenital discomfort, and weight loss.

- Past medical history related to haematological, oncologic, endocrine, cardiovascular, and infectious conditions — to identify risks associated with **oral corticosteroid use** and potential complications of **immunosuppressive therapy**.
- Assessment of pregnancy potential and current contraceptive practices, especially if immunosuppressive therapy is planned.
- Review of recent medication use that may induce pemphigus, including:
 - ACE inhibitors
 - Angiotensin receptor blockers
 - Beta-blockers
 - Cephalosporins
 - Phenylbutazone
 - Pyritinol
 - Thiopronine
 - Rifampicin

(Note: D-penicillamine, a classic cause of drug-induced pemphigus, is now rarely used.)

Consider discontinuation of suspected drugs if there is a clear temporal relationship between drug initiation and lesion onset.

- Evaluation of quality-of-life impact using validated questionnaires such as **DLQI**, **ABQOL**, or **TABQOL**.
- Verification of vaccination status. Inquire about planned travel to regions requiring live vaccines — these should be administered **before** starting immunosuppressive therapy.

Physical examination

General examination

It is recommended to assess:

- Extent of skin and mucous membrane involvement.
- Degree of mucosal damage and associated functional impairments:
- Dysphagia
- Dysphonia
- Weight loss
- Visual disturbances
- Dyspareunia

It is recommended to quantify skin and mucosal lesion burden using one of the two validated scoring systems:

Pemphigus Disease Area Index (PDAI) or **Autoimmune Bullous Skin Intensity and Severity Score (ABSIS)**.